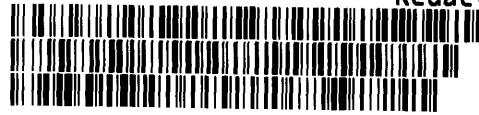


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Final Surgical Pathology Report

Procedure:

Diagnosis

- A. Breast, left, segmental mastectomy: Invasive ductal carcinoma, grade 3, size 2.8 cm in greatest dimension; negative margins of excision.
- B. Sentinel lymph node #1, biopsy: Two negative lymph nodes (0/2).
- C. Sentinel lymph node #2, biopsy: One negative lymph node (0/1).
- D. Breast, medial to left segmental mastectomy, biopsy: Negative for malignancy or atypia.
- E. Breast, inferior to left segmental mastectomy, biopsy: Negative for malignancy or atypia.
- F. Breast, posterior to left segmental mastectomy, biopsy: Skeletal muscle; negative for malignancy or atypia.
- G. Breast, lateral to left segmental mastectomy, biopsy: Negative for malignancy or atypia.
- H. Breast, superior to left segmental mastectomy, biopsy: Negative for malignancy or atypia.

Microscopic Description:

Invasive Carcinoma:

Histologic type: Ductal
Histologic grade: 3
Overall grade: 9/9
Architectural score: 3
Nuclear score: 3
Mitotic score: 3
Greatest dimension (pT): 2.8 cm
Specimen margins: Negative
Vessel invasion: Not identified
Calcification: Present

ICD-0-3

carcinoma, infiltrating duct, NOS
8500/3

Site: Breast, NOS C50.9

fw
10/27/12

Ductal carcinoma in situ:

Histologic pattern: Solid
Nuclear grade: 3
Central Necrosis: Present
% DCIS of total tumor (if mixed): 1%
Extensive intraductal component (present/absent): Absent
Specimen margins: Negative
Calcification: Negative

Description of non-tumorous breast: Fibrocystic changes, with ductal hyperplasia of usual type, a minute fibroadenoma, and apocrine metaplasia, with calcification.

Comments: Sentinel lymph nodes negative, confirmed by negative cytokeratin immunostaining of blocks BFS1 and CFS2. Cytokeratin staining was also performed on one block from part E, to exclude involvement by carcinoma. Prior biopsy site identified. Extensive central tumor necrosis is present.

Prognostic markers: See core biopsy report,

4x8, 14x2, 15x2, 20x3

[A few of the antibodies used in our laboratory may be classified as analyte specific reagents. These antibodies are monitored and controlled in our laboratory and their performance for in vitro diagnosis is well described in the medical literature. They have not been cleared or approved by the FDA.]

Specimen

- A. Left Breast
- B. Left axillary sentinel lymph node #1, hot, blue,
- C. Left axillary sentinel lymph node #2, not hot, not blue, but palpable
- D. Medial to segmental mastectomy left breast
- E. Inferior to segmental mastectomy left breast
- F. Posterior to segmental mastectomy left breast
- G. Lateral to segmental mastectomy left breast
- H. Superior to segmental mastectomy left breast

Clinical Information

Left breast cancer

Intraoperative Consultation

- B) Left axillary sentinel node #1: Two negative lymph nodes (0/2).
- C) Left axillary sentinel node #2, biopsy: One negative lymph node (0/1).

Gross Description

- A. Received unfixed for tissue procurement, labeled left breast segmental mastectomy single long lateral, double long anterior, is a 100 gram breast segmental excision that is 9 cm from anterior to posterior, 7.5 cm medial to lateral, and 3 cm from superior to inferior. The superior surface is inked black, and a section is submitted for tissue procurement as requested. An additional section of uninvolved fatty tissue is submitted from the posterior aspect, for procurement. Additional sections after fixation. The superior surface is inked black, medial green, lateral yellow, inferior blue. There is a tan, firm, lobulated mass lesion in the mid-portion of the specimen, 3.0 x 2.0 x 1.7 cm. This is 3 mm from the green-inked medial margin (closest margin). All other margins are grossly greater than 1 cm from the mass. Representative sections: 1 Anterior margin, 2 adjacent section with margin, 3/4/5 contiguous tumor, possible biopsy cavity in 5, 6/7/8 contiguous, 9-10 random medial and lateral, possible close margin, 11-14 additional posterior sections, with last section = posterior margin.
- B. Received unfixed for frozen section, labeled left axillary sentinel node #1, are 2 adjacent and attached lymph nodes, 0.9 cm in greatest dimension each, bisected and entirely submitted in blocks B1 and B2.
- C. Received unfixed for frozen section, labeled left axillary sentinel node #2, is a single lymph node, 1.4 x 1.3 x 1.0 cm, bisected and entirely submitted in 2 blocks.
- D. Received fresh and subsequently fixed in formalin labeled "medial to segmental mastectomy left breast" is a 5.2 x 3.5 x 1.5 cm yellow lobular fatty tissue fragment which has a suture designating the new medial border. The surface is inked and the specimen is sectioned to show yellow lobular fatty cut surface with minimal fibrous tissue present. No discrete residual tumor is grossly identified. The specimen is entirely submitted labeled 1 through 10.
- E. Received fresh and subsequently fixed in formalin labeled "inferior to segmental mastectomy left breast" is a 4.3 x 2.7 x 2.2 cm yellow lobular fatty tissue fragment which has a suture designating new inferior border. The surface is inked and the specimen is sectioned to show yellow lobular fatty cut surface with minimal fibrous tissue present. No residual tumor is grossly identified. The specimen is entirely submitted labeled 1 through 9.

- F. Received fresh and subsequently fixed in formalin labeled "posterior to segmental mastectomy left breast "is a 3.2 x 2.5 x 1.0 cm yellow lobular fatty tissue fragment which is partially covered with skeletal muscle. There is a suture designating the new posterior margin. The surface is inked and the specimen is sectioned to show a yellow lobular fatty cut surface with scanty fibrous tissue present. The specimen is entirely submitted in 5 cassettes.
- G. Received fresh and subsequently fixed in formalin labeled "lateral to segmental mastectomy left breast "is a 4.3 x 3.5 x 1.5 cm yellow lobular fatty tissue fragment. There is a suture designating the new lateral border. The surface is inked and the specimen is sectioned to show yellow lobular fatty cut surface with minimal fibrous tissue present. No residual tumor is grossly identified. The specimen is entirely submitted in 9 cassettes.
- H. Received fresh and subsequently fixed in formalin labeled "superior segmental mastectomy left breast "is a 4.3 x 2.2 x 1.0 cm yellow lobular fatty tissue fragment which has a suture designating the new superior border. This is inked and the specimen is sectioned to show yellow lobular fatty cut surface with scanty fibrous tissue present. The specimen is entirely submitted in 4 cassettes.

Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
History Discrepancy		
Pathology Discrepancy		
Specimen Discrepancy		
Consent		
Date Reviewed		

10/27/12